Case D

Mr. Rossi — 45M, Amateur Marathon Runner | 3-month deep left buttock pain · "Tight hamstrings" · Occasional cramp to knee · Worse: hard chair sitting, initial running phase

Target: 70%+ | Musculoskeletal (MSK) Physiotherapy

■ PROBLEM LIST

- Deep gluteal pain left-sided
- Piriformis/deep external rotator overload
- Sciatic nerve mechanosensitivity
- Tenderness deep to gluteus maximus
- Pain on resisted hip abduction (seated)
- Possible hip internal rotation restriction
- Unable to run comfortably (initial phase pain)
- Cannot sit on hard chairs comfortably
- Likely impaired marathon training plan
- Possible impact on work (prolonged sitting)
- Recreational activity restriction

■ RED FLAGS

None identified

■ PATHOPHYSIOLOGY

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■ DIFFERENTIAL DIAGNOSES

1. L5/S1 Lumbar Radiculopathy

- Buttock pain radiating to leg in a runner (disc loading with high-impact sport)
- Leg pain with cramping quality
- Supporting: 45-year-old male (age-related disc degeneration possible)

2. Proximal Hamstring Tendinopathy

- Ischial tuberosity / buttock pain in a runner
- Worse sitting on hard surfaces (ischial tuberosity compression)
- Onset with increased training load

■ OBJECTIVE ASSESSMENTS & HOW TO PERFORM

FADIR Test (Flexion, Adduction, Internal Rotation)

Test 1: FADIR Test (Flexion, Adduction, Internal Rotation) What it measures: In the context of DGS, FADIR elongates and compresses the piriformis against the sciatic nerve. It can also test for hip impingement (FAI), but the location of pain provoked distinguishes the two (groin

FAIR Test (Flexion, Adduction, Internal Rotation — Resisted Variant / Beatty Test)

Test 2: FAIR Test (Flexion, Adduction, Internal Rotation — Resisted Variant / Beatty Test) What it measures: Isometric piriformis activation under stretch — specifically isolates the piriformis as the compressing structure. More specific for piriformis syndrome than the passive F

Neural Tension Test — Slump Test (Modified for Peripheral Sensitisation)

Test 3: Neural Tension Test — Slump Test (Modified for Peripheral Sensitisation) What it measures: Sciatic nerve mechanosensitivity and the degree to which neural tension contributes to symptoms. Differentiates between a "tight hamstring" (no neural sensitisation) and a mechanose

Technique: Deep Soft Tissue Massage / Trigger Point Release to Piriformis + Sciatic Nerve Mobilisation (Hands-On)

Technique: Deep Soft Tissue Massage / Trigger Point Release to Piriformis + Sciatic Nerve Mobilisation (Hands-On) Rationale: Direct sustained pressure (ischaemic compression) to the piriformis muscle belly reduces myofascial trigger point activity, decreases muscle tone, and indi

■ TREATMENT PLAN

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